

Anesthesia Record

Pre-Operative Evaluation

Case Identification

Name

Janice Davids

Procedure Name

Upper Arch All-on-X inp

Date

Friday, May 22, 2026

DOB

Not Specified

Weight

63 kg

Anesthesiologist

Dr. Joseph Lee

Surgeon

Dr. Matthews

Assistant

Haley Garcia

Clinical Assessment

History

63yoF with pmhx significant for mild asthma and HTN presenting for All on X surgery under GA. Patient denies any recent URI, cough, cold, congestion, fevers, chills, chest pain, SOB, DOE, dysuria, abdominal pain, nausea, vomiting. METS > 4.

NPO for > 8hours.

Allergies

NKDA

Medications

• Lisinopril 20mg (QD)

Physical Exam & Airway

ASA Class

ASA 2

General Appearance

Anxious

Resp

CTAB

CV

S1 S2 WNL

Neuro

No Focal or Gross Deficits

Mallampati

Class 2

Mouth Opening

Normal

Dentition

Poor, Dentures/Bridge

Anesthesia Plan

Anesthesia Type

GA

Airway Plan

ETT (nasal)

Post-Op Plan

Home w/ Supervision

Procedure Details

Intra-Operative Record

IV Access

12:21 PM

Line	Gauge	Location	Placed By
#1	22G	R Hand	Anesthesiologist

Airway Management

12:25 PM

Device	Nasal Side	ETT Size	Laryngoscopy
Nasal ETT	Right	7.0	Direct
Blade	Grade of View	Attempts	Position
Mac 3	2a	1	21 cm @ Nares
Breath Sounds			
Equal + Bilateral			

Comments

Eyes taped and protected prior to procedure. Nares pretreated with oxymetazoline; pharynx suctioned and noted to be free of secretions. Endotracheal tube warmed slightly to facilitate atraumatic insertion. Standard induction achieved with uneventful mask ventilation. Laryngoscopy performed with clear visualization of glottic structures. ETT advanced smoothly through the treated nares and vocal cords without resistance or trauma. No evidence of bleeding or injury to the nares or airway. Cuff inflated to appropriate pressure; bilateral breath sounds and EtCO₂ confirmed. ETT secured with tape; head roll placed to maintain midline positioning. Patient tolerated intubation without complication.

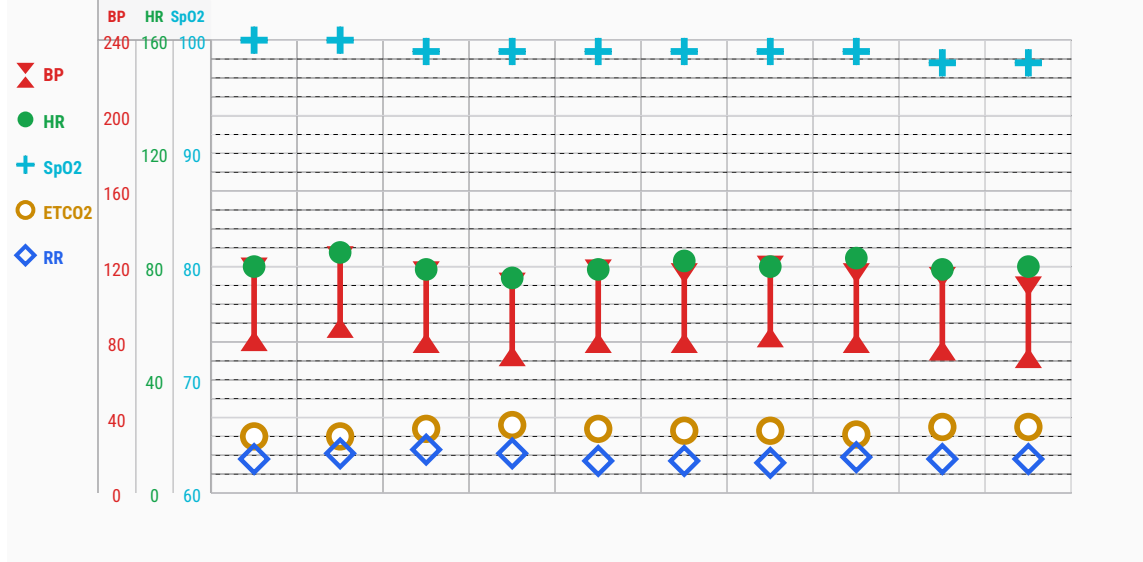
Extubation

1:45 PM

Oropharynx suctioned and clear of secretions. Patient emerged from anesthesia with intact cough and gag reflexes. Extubated smoothly while hemodynamically stable and breathing spontaneously. Airway maintained independently with appropriate oxygen saturation. No signs of laryngospasm, bronchospasm, or airway obstruction observed.

Date
5/22/2026

Medications & Local Anesthesia										
Dexamethasone			8 mg							
Fentanyl		25 mcg								
Propofol		200 mg								
Rocuronium		20 mg								



MAC		0.8	0.9	1.0	1.0	1.0	0.8	0.9	0.9	0.9	
Vent Mode	Spont	Spont	Spont	Spont	Spont	Spont	Spont	Spont	Spont	Spont	
Position	Supine	Supine	Supine	Supine	Supine	Supine	Supine	Supine	Supine	Supine	
Events	START IV	AIRWAY TRIN	Pro.Start							COMMENT	

ANESTHESIA START 12:21 PM

IV 12:21 PM

AIRWAY PLACEMENT 12:25 PM

THROAT PACK IN 12:25 PM

PROPOFOL 200 MG 12:25 PM

FENTANYL 25 MCG 12:25 PM

ROCURONIUM 20 MG 12:25 PM

PROCEDURE START 12:30 PM

DEXAMETHASONE 8 MG 12:30 PM

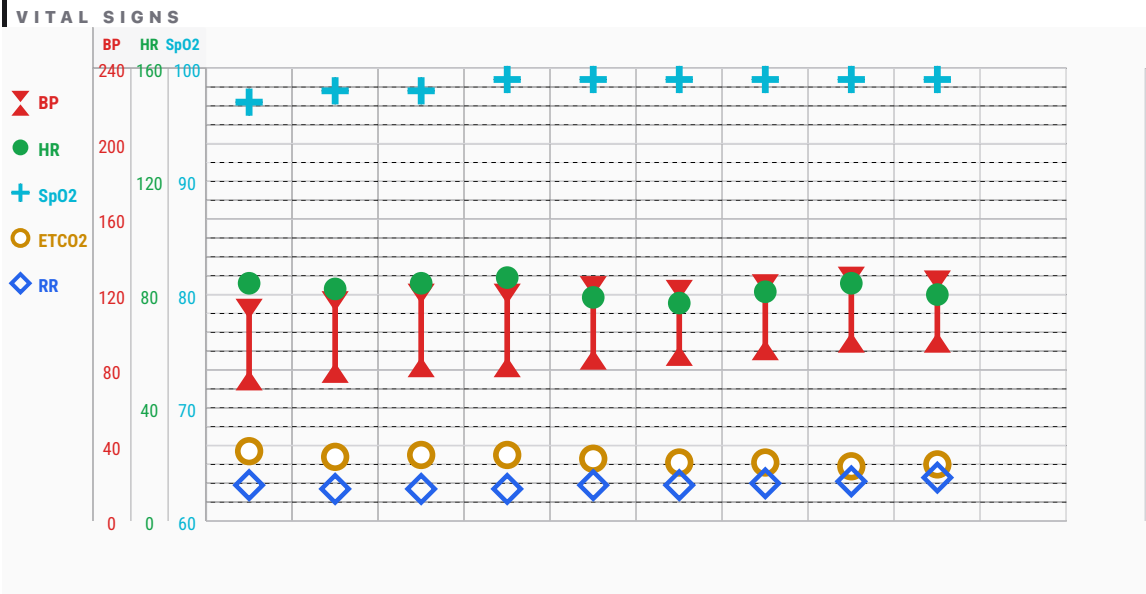
COMMENT 1:05 PM

Zygomatic implants placed bilaterally. Moving onto anterior implants. Digital lab at bedside to guide placement.

	1:10 PM	1:15 PM	1:20 PM	1:25 PM	1:30 PM	1:35 PM	1:40 PM	1:45 PM	1:50 PM	TOTAL
GASES										
O2 (L/min)	2						7			
Sevoflurane (%)	2				1.2					

FLUIDS										
LR	550									950 mL

MEDICATIONS & LOCAL ANESTHESIA										
Acetaminophen					1000 mg					1000 mg
Fentanyl			25 mcg							25 mcg
Ketorolac			15 mg							15 mg
Ondansetron						4 mg				4 mg



CLINICAL DATA										
MAC	0.8	0.8	0.8	1.0	0.7	0.6	0.3	0.1	0.0	
Vent Mode	Spont	Spont	Spont	Spont	Spont	Spont	Spont	Spont	Spont	
Position	Supine	Supine	Supine	Supine	Supine	Supine	Supine	Supine	Supine	
Events					COMMENT		Pro.End TP. OUT COMMENT	EXT. → H.O.	STOP	

EVENT LEGEND

FENTANYL 25 MCG

1:20 PM

KETOROLAC 15 MG

1:20 PM

ACETAMINOPHEN 1000 MG

1:20 PM

COMMENT

1:30 PM

Implant placement complete, lab to take final scans and closure to proceed as anticipated.

ONDANSETRON 4 MG

1:35 PM

PROCEDURE END

1:40 PM

THROAT PACK OUT

1:40 PM

COMMENT

1:40 PM

Procedure complete. Throat pack removed and oropharynx suctioned. Hemostasis adequate.

EXTUBATION

1:45 PM

TRANSPORT

1:45 PM

HANDOFF

1:45 PM

ANESTHESIA STOP

1:50 PM

Additional Data

Vitals Table

Time	BP (SBP/DBP)	HR	SpO2	ETCO2	RR	MAC
12:20 PM	120/80	80	100	30	18	—
12:25 PM	126/87	85	100	30	21	0.8
12:30 PM	118/79	79	99	34	23	0.9
12:35 PM	112/72	76	99	36	21	1.0
12:40 PM	119/79	79	99	34	17	1.0
12:45 PM	117/79	82	99	33	17	1.0
12:50 PM	121/82	80	99	33	16	0.8
12:55 PM	117/79	83	99	31	19	0.9
1:00 PM	115/75	79	98	35	18	0.9
1:05 PM	110/71	80	98	35	18	0.9
1:10 PM	113/74	84	97	37	19	0.8
1:15 PM	117/78	82	98	34	17	0.8
1:20 PM	121/81	84	98	35	17	0.8
1:25 PM	121/81	86	99	35	17	1.0
1:30 PM	125/85	79	99	33	19	0.7
1:35 PM	123/87	77	99	31	19	0.6
1:40 PM	126/90	81	99	31	20	0.3
1:45 PM	130/94	84	99	29	21	0.1
1:50 PM	128/94	80	99	30	23	0.0

Recovery

Discharge Vitals

Aldrete

9

Discharge Ready

SpO₂

100 %

HR

76 bpm

SBP/DBP

126/68 mmHg

RR

25 /min

Temp

98.2 °F

Case Timeline

Start

12:21 PM

End

1:50 PM

Duration

1:29

Controlled Substance Reconciliation

Medication

Total Given

Amount Wasted

Fentanyl

50 mcg

50 mcg

Provider Signature



Witness Signature



Wasted By

Witnessed By

✓ Reconciliation Verified

Discharge Attestation

Patient denies any nausea or vomiting. She is comfortable with a 2/10 pain level. she is alert and oriented times three and has achieved adequate hemostasis. Her oxygen saturation is at 100% on room air and her vitals are within 20% of baseline. I handed off care to the surgical team and recovery nurse.

Electronically Signed By

Dr. Joseph Lee

Timestamp

5/22/2026 12:37 PM